

NHS RTT Performance — A Management Case Study

A service improvement perspective on referral-to-treatment (RTT) performance, developed from dashboard analysis

Problem Statement

Only 64% of referred patients are currently being treated within the NHS 18-week referral-to-treatment (RTT) target — well below the national standard. Performance is not evenly distributed: Orthopaedics and Ophthalmology account for a disproportionate share of breaches, and routine-priority patients are breaching at a far higher rate (41%) than urgent (26%) or cancer-pathway (13%) patients.

In short: more than a third of patients are waiting too long to be treated, concentrated in two specific specialties, while urgent and cancer pathways continue to be appropriately protected.

Root Cause Analysis

The pattern suggests this is a capacity issue, not a prioritisation failure. The system is correctly fast-tracking urgent and cancer cases — the breach concentration in routine patients reflects where they sit in the queue when capacity is constrained, not clinical neglect.

Orthopaedics and Ophthalmology are specialties known nationally for longer waits due to theatre and clinic capacity constraints relative to referral demand. This points to a structural mismatch between referral volume and available capacity in these two departments specifically, rather than a hospital-wide problem.

Recommendations

1. **Run a demand-and-capacity review** for Orthopaedics and Ophthalmology — measure actual referral volume against current clinic and theatre capacity, rather than relying on historical scheduling patterns.
2. **Make the long-wait backlog a standing weekly review**, not a static report. Proactively contact patients approaching 16+ weeks before they breach, rather than only reviewing breaches after they occur.
3. **Introduce waiting list validation** — a recognised NHS technique of contacting long-waiting patients to confirm continued need for treatment. This frequently reduces apparent backlog size without requiring additional capacity.
4. **Consider Patient-Initiated Follow-Up (PIFU)** for low-risk routine patients in the worst-performing specialties, replacing

automatic routine follow-ups with patient-initiated contact when needed — freeing capacity for new referrals.

How I Would Measure Success

- **Overall breach rate:** track the reduction from 36% toward the NHS operational standard (85%+ compliance), as the primary lagging indicator of progress.
- **Patients at 16+ weeks (leading indicator):** monitor this group weekly as an early warning signal, allowing intervention before a breach occurs rather than reporting on it afterward.
- **Breach rate by specialty:** review Orthopaedics and Ophthalmology monthly to confirm whether the capacity review and interventions are shifting the two worst-performing areas specifically, not just the hospital-wide average.

This case study was developed from a synthetic Power BI dashboard analysis of RTT performance data, built to demonstrate data-driven service improvement thinking relevant to NHS patient pathway and administration roles.